ferried out of the ambulance for mandatory electric cremation. The whole experience was mortifying—unheard of and unseen.

In MP's case, the Maharashtra state government along with the MCGM should have ensured that he received appropriate oxygen support and medical care instead of announcing a blanket ban on private hospitals treating COVID-19 positive patients prematurely. They should have strengthened their own infrastructure first without shoving patients in facilities which weren't even equipped with ventilators or ICUs. Several lives could have been saved if it wasn't for such instances of policy paralysis in many state governments. However, a crisis like this was going to be beyond the scope of a state government and would turn out to be highly challenging even for a competent central government. Countries simply did not have at their ready disposal the tools to fight an unknown biological enemy. Very few governments are capable of building new hospital structures on an emergency basis. And even fewer have the manufacturing companies to make new medical supplies in quantities which India's projected need was. However, after the initial shock of COVID-19 fatalities, many states set up state-specific COVID-19 task forces and specific teams of experts to help form state response guidelines.

As case studies of such episodes made their way to the ears of the Health Department and the PMO, there was agreement that although health is a state subject, in a pandemic of such sort, the Centre would have to lead with detailed guidelines, policies and orders.